**This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.**

Patient Group Direction
for the supply of Tamsulosin 400mcg MR capsules for the treatment of Benign Prostatic Hyperplasia (BPH)

Patient Group Direction, version 006, issued 30 September 2026. Supersedes Version 005, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

## Summary of NICE / NICE CKS Guidelines for Benign Prostatic Hyperplasia (BPH)

### Overview

- **Benign Prostatic Hyperplasia (BPH):** A common condition in older men characterised by enlargement of the prostate gland.
- **Lower Urinary Tract Symptoms (LUTS):** BPH causes LUTS including frequency, urgency, nocturia, poor stream, hesitancy, and incomplete emptying.
- **Epidemiology:** Primarily affects men aged over 50 years, with prevalence increasing with age.

### Assessment of BPH

- **IPSS Score:** International Prostate Symptom Score used to quantify symptom severity (mild, moderate, or severe).
- **Digital Rectal Examination (DRE):** Perform if appropriate to assess prostate size and consistency.
- **Prostate-Specific Antigen (PSA):** Consider testing if indicated and after informed discussion with patient about benefits and limitations.

- **Renal Function: check where renal impairment secondary to obstruction is suspected. This PGD does not require an eGFR result; suspected renal impairment from obstruction is a red flag and a referral.**
- **Urinary Flow Rate:** Non-invasive objective measure of obstruction severity.
- **Post-Void Residual (PVR):** Measure of urine remaining in bladder after micturition; elevated PVR suggests retention risk.

**Management of BPH**

- **Watchful Waiting:** Initial conservative approach for mild symptoms; includes lifestyle modifications and regular monitoring.
- **Alpha-blockers:** First-line pharmacotherapy for symptomatic relief of LUTS in BPH; tamsulosin is a selective alpha-1a adrenoceptor antagonist.
- **5-Alpha Reductase Inhibitors:** Indicated for men with large prostate glands; slow disease progression and reduce prostate volume over time.

**Red Flags Requiring Specialist Referral**

- **Acute Urinary Retention:** Inability to void spontaneously; immediate specialist assessment required.
- **Recurrent Urinary Tract Infections:** May indicate incomplete bladder emptying or other complications.
- **Haematuria:** Blood in urine requires investigation to exclude other pathology.
- **Renal Impairment:** Evidence of renal dysfunction secondary to urinary obstruction.
- **Elevated PSA:** Concern for malignancy requires specialist evaluation.

## Patient Group Direction

for the supply of

**Tamsulosin 400mcg MR capsules**

for the treatment of

**Benign Prostatic Hyperplasia (BPH)**

**Healthcare professionals covered and training**

| | **Healthcare professionals allowed to work under this PGD and requirements** |
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| **Qualifications and Registration** | • Pharmacist registered and practicing with the GPhC<br>• Pharmacy technician registered and practicing with the GPhC |
| **Training and assessment** | Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team.<br>• All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines<br>• All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e- |

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| | learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required<br>• All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.<br>• Must work in compliance with the SOPs of their own employer<br>• All users must have appropriate indemnity in place to cover this service |
| **Further training and responsibilities** | • All users must be up to date with CPD requirements and appraisal<br>• All users must be up to date with MHRA and safety alerts with regards to medical products<br>• All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005. |

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| **PGD Indication** | Symptomatic relief of lower urinary tract symptoms (LUTS) associated with benign prostatic hyperplasia (BPH) in adult men |
| **Inclusion criteria** | Male patients aged 45 years and above<br>Clinical symptoms consistent with BPH (frequency, urgency, nocturia, poor urinary stream, hesitancy, incomplete bladder emptying)<br>IPSS (International Prostate Symptom Score) of moderate severity or above (≥8)<br>Informed consent obtained after discussion of benefits, risks, and alternatives<br>Blood pressure measured lying and standing at every supply, first and repeat, with both readings recorded and neither exclusion threshold below met |
| **Exclusion criteria** | Known hypersensitivity to tamsulosin or any excipients in the formulation<br>Orthostatic hypotension: a history of blood pressure drop on standing, or a fall in systolic blood pressure of 20 mmHg or more on |

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| | standing measured at this consultation (lying or seated for at least 5 minutes, then standing for 1 to 3 minutes)<br><br>Severe hepatic impairment (Child-Pugh Grade C)<br><br>Concurrent use of other alpha-1 adrenoceptor antagonists<br><br>Planned cataract or glaucoma surgery (due to risk of intraoperative floppy iris syndrome - IFIS)<br><br>Uncontrolled hypertension: a blood pressure of 160/100 mmHg or above (systolic 160 or above, or diastolic 100 or above), lying or standing, measured at this consultation, whether or not the patient is on treatment. Refer to the GP<br><br>Acute urinary retention requiring catheterisation or in-patient management<br><br>Visible or non-visible haematuria (refer: urological investigation before any treatment of symptoms)<br><br>Current or recurrent urinary tract infection, or dysuria with fever (refer)<br><br>Palpable bladder, or symptoms suggesting chronic retention: overflow incontinence, a constant feeling of incomplete emptying with a poor stream (refer for post-void residual measurement)<br><br>Known or suspected prostate cancer, an abnormal digital rectal examination, or a raised PSA (refer)<br><br>Neurological disease affecting bladder function: multiple sclerosis, Parkinson's disease, spinal cord disease, diabetic neuropathy (refer)<br><br>Aged under 45: lower urinary tract symptoms at this age are unlikely to be BPH; refer to the GP for diagnosis<br><br>Symptoms not previously assessed by a GP or urologist, unless the GP is informed on the day of supply and the patient agrees to attend the GP within 6 weeks for examination and, where indicated, PSA testing |
| **Cautions** | Orthostatic hypotension: especially in elderly patients or those on concurrent antihypertensive medication; patients should be advised to sit or lie down if dizziness occurs<br><br>Renal impairment: no dose adjustment is needed (SmPC); the SmPC advises caution below eGFR 10 mL/min/1.73m². Renal function is not measured under this PGD; a patient with known severe renal impairment or on dialysis is referred.<br><br>Hepatic impairment: avoid in severe hepatic disease; use with caution in mild to moderate impairment<br><br>Concurrent use of PDE5 inhibitors (sildenafil, tadalafil): additive hypotensive effects; monitor blood pressure carefully<br><br>History of syncope or fainting: increased risk with tamsulosin; consider discontinuation if syncope occurs |

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| | Surgery: inform all healthcare professionals, including ophthalmologists, of tamsulosin use before any planned procedures |
| **Actions if patient is excluded or declines treatment** | Advise on alternative treatment options and how these can be accessed.<br><br>Document any advice given and the decision reached. Inform or refer to the GP as appropriate. |

***Details of the medicine***

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| **Name, form and strength of medicine** | Tamsulosin 400 micrograms modified-release capsules |
| **Legal category** | POM (Prescription Only Medicine) |
| **Storage** | Store below 25°C. Protect from light and moisture. Keep in original container. |
| **Route/method of administration** | Oral. Modified-release capsule to be swallowed whole with water after food (preferably after breakfast). Not to be crushed, chewed, or opened. |
| **Dose and frequency** | 400 micrograms once daily<br><br>Take after food, preferably with breakfast<br><br>Modified-release capsule must be swallowed whole - do not crush, chew, or open the capsule |
| **Quantity to be administered and/or supplied** | 28 capsules per supply (4 weeks). No larger supply under this PGD. |
| **Maximum or minimum treatment period** | Maximum under this PGD: an initial supply of 4 weeks, then, where the IPSS has improved by 3 points or more at the 4 to 6 week review and the patient has been examined by the GP since the first supply under this PGD (an assessment before the first supply does not count), further supplies of 28 capsules (4 weeks) each to a maximum of 12 months' continuous treatment, after which the GP takes over prescribing. No improvement at 4 to 6 weeks, or any new exclusion, ends supply under this PGD and the patient is referred.<br><br>Blood pressure is measured lying and standing at every supply, first and repeat, and both readings recorded. A reading of 160/100 mmHg or above, or a postural systolic fall of 20 mmHg or more, ends supply under this PGD and the patient is referred. |
| **Adverse effects** | Very Common (≥1 in 10): Abnormal ejaculation (predominantly retrograde ejaculation)<br><br>Common (1 in 100 to 1 in 10): Dizziness, headache, rhinitis (nasal symptoms), palpitations, anxiety |

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| | Uncommon (1 in 1,000 to 1 in 100): Postural/orthostatic hypotension, syncope (fainting), nausea, constipation, diarrhoea, asthenia (weakness), rash, pruritus (itching)<br><br>Rare (<1 in 1,000): Priapism (prolonged erection >4 hours), intraoperative floppy iris syndrome (IFIS) during cataract/glaucoma surgery, Stevens-Johnson syndrome, angioedema |

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| **Yellow card reporting** | Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate. |
| **Records to be kept** | Record the following information:<br>• that valid informed consent was given<br>• name of individual, address, date of birth and GP<br>• name of healthcare practitioner<br>• name and brand of medication<br>• date of supply dose, form and route quantity supplied<br>• blood pressure measured at this supply, lying and standing, both readings and the postural change in systolic pressure<br>• advice given, including advice given if excluded or declines treatment<br>• details of any adverse drug reactions and actions taken supplied via PGD<br>Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made.<br><br>A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows:<br>• Adults aged 18 years and over - keep records for 8 years<br>• Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday) |

***Patient information***

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| **Written information to be given to patient or carer** | Supply the patient information leaflet (PIL) provided with the medication and discuss key safety points verbally. |
| **Follow-up advice to be given to patient or carer** | Take the capsule after food, preferably with breakfast, to minimise side effects<br><br>Stand up slowly from sitting or lying down, especially at the start of treatment or after a dose increase, as dizziness may occur<br><br>Inform any healthcare professional (including eye surgeons and dentists) that you are taking tamsulosin before any planned procedures, especially eye surgery |

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| | Report any episodes of dizziness, fainting, or lightheadedness to your doctor immediately - these may require dose adjustment or discontinuation<br><br>Inform your GP or pharmacist if you experience abnormal ejaculation or other sexual dysfunction during treatment<br><br>Do not crush, chew, or open the capsule - swallow whole to maintain the modified-release formulation<br><br>An erection lasting longer than 4 hours (priapism) is an emergency: go to A&E immediately<br><br>Seek urgent medical attention if you experience rapid heartbeat, chest pain, or severe dizziness<br><br>Avoid sudden changes in posture (e.g., rapid standing from lying down) to minimise orthostatic hypotension<br><br>If you develop a rash or suspect an allergic reaction, discontinue the medication and contact your GP or pharmacist<br><br>Review appointment arranged for 4-6 weeks to assess symptom improvement and tolerability<br><br>If you have any concerns about the medication or experience unexpected symptoms, contact your GP or pharmacist |

**Key references**


- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Lower Urinary Tract Symptoms in Men
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines


**Agreement to practise by the Registered Healthcare Professional**

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

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| Name and address of pharmacy premises / clinic premises to which this PGD relates | | | |
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| Name of healthcare | Job title | Registration number | Signature |

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| Valid from date | Expiry date | | |
| 30/9/26 | 31/7/27 | | |

**PGD adoption and authorisation by the employer/clinical lead**

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

| Name of superintendent / clinical lead | Job title & name of organisation | Signature | Date |
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| | | | |
| Name of professional group signatory | Job title & name of organisation | Signature | Date |
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**Change history to version 001**

Later versions are recorded in the Version and change record at the end of this document.

| **Version number** | **Change details** | **Date** |
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| 001 | Development & issue of new PGD | 1st November 2025 |
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**Version and change record**

Version: v006

Issued: 30 September 2026

Supersedes: Version 005, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

**Previous version records**

Version: v005

Issued: 14 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 47: blood pressure is now measured lying and standing at every supply, first and repeat, with both readings recorded; the exclusions state the thresholds, 160/100 mmHg or above or a postural fall in systolic pressure of 20 mmHg or more, in place of the unmeasured history questions, and the inclusion criteria, treatment period row and records row carry the requirement.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document

above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Cover and arm heading corrected from administration to supply.

Inclusion age is 45 and above, matching the under 45 exclusion; it said 18.

Quantity and maximum treatment period reconciled: 28 capsules (4 weeks) per supply throughout; the maximum period row allowed 12 week supplies.

Continuation requires a GP examination since the first supply under this PGD; the row did not say whether an earlier assessment counted.

Renal caution and the guidance summary renal line state that no eGFR result is required and what is referred.

Priapism advice is go to A&E immediately, matching the ED PGD.

Broken NICE MPG2 link corrected.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v002

Issued: 11 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Exclusions added: haematuria, current or recurrent UTI, chronic retention, known or suspected prostate cancer, neurological bladder disease, age under 45, and symptoms never assessed by a GP without a GP appointment within 6 weeks. The previous version's summary named these as red flags but the exclusion criteria did not.

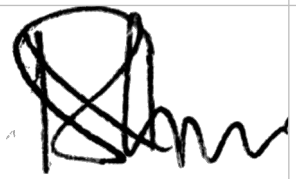
A maximum treatment period is stated: 4-week initial supply, continuation only after GP examination and IPSS improvement, 12 months' maximum before GP prescribing. The previous version said 'ongoing treatment'.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature

blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

## Signed on behalf of Get Real Health

Version v006, 30 September 2026.

| Name | Qualification | Signature | Date |
|---|---|---|---|
| Nitin Shori | Medical Director<br>GMC: 6047293 |  | 30 September 2026 |
| Chris Pilkington | Head Pharmacist<br>GPhC: 2046322 |  | 30 September 2026 |

*Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.*